

CH SHARIF AND SAEED HOSPITAL

FULL Client Functional Walkthrough v7.2

Detailed client-facing walkthrough of every revised v7.2 business flow from admission to provider settlement.

FINAL SOURCE OF TRUTH - VERSION 7.2

Admission begins at Front Desk. Department bills remain separate. Central collections are allocated department-wise. Doctor discharge re-authentication, outsourced department settlement, Panel receivables, high-cost medicine approval, doctor-sponsored discount and doctor tax rules are included.

Supersedes v7.1 for all revised admission, billing, department settlement, doctor finance and panel-flow rules.

1. System at a Glance

The HMS keeps five hospital portals and a separate standalone Pharmacy project. v7.2 moves admission creation to Front Desk and formalizes separate department billing, central collections, outsourced-provider settlement, doctor discharge credentials, doctor tax and Panel receivables.

2. Front Desk Admission

Patient arrives at Front Desk. Staff selects Self-Pay or searches permanent Panel Patient Registry. Admission file is created with doctor, department, bed preference and initial fulfillment choices. Optional advance can be collected immediately.

Front Desk field	Example contents
Patient Type	Self-Pay / Panel
Admitting Doctor	Doctor from Staff Master
Department	Admitting clinical department
Advance	Optional amount/method
Fulfillment Choices	Configured department Hospital Managed/Internal vs Self/External

3. Advance Meaning

Admission advance is patient credit. It is not immediately counted as Lab, Pharmacy or Hospital revenue until it is allocated against actual department invoices.

4. Admission Handoff

After admission creation, the file appears in Admission Portal. Admission staff manages bed, stay, procedures, department requests and running services but does not receive Hospital patient cash.

5. Department Fulfillment Modes

Hospital may configure service departments such as Pharmacy, Lab or Neurology to allow Hospital Managed/Internal or Self/External fulfillment. The choice is recorded and can be changed with reason/history if policy allows.

6. Separate Department Bills

Hospital Services, Lab, Pharmacy, Neurology and every other bill-owning department keep separate invoices. A consolidated patient statement is only a presentation layer; it does not merge ownership.

7. Running Bill

During admission, charges accumulate by department. Patient/attendant can request a current statement showing each department subtotal, discount/contract effect, paid amount and outstanding.

8. Partial Payment

Patient can pay during the stay. Front Desk enters the payment and allocates it across department invoices. Allocation total must reconcile to the posted payment, or remain explicit patient credit if policy allows.

Department	Outstanding	Allocated now
Hospital	25,000	10,000
Lab	12,000	5,000
Pharmacy	18,000	5,000

9. Hospital Discount

Hospital-funded discount reduces Hospital Services only. It does not silently reduce Lab, Pharmacy or outsourced provider invoices.

10. Doctor-Sponsored Discount

When a commission doctor personally gives their patient a discount, it is tagged Doctor-Sponsored Discount. The patient service charge is reduced and the discount is charged against that doctor commission payable.

11. Doctor Commission

Commission rules remain service-specific and doctor-specific. Commission can be Fixed or Percentage and can use Gross or Net After Discount base according to contract.

12. Doctor Commission Tax

Commission tax is configured separately on the doctor profile with effective-dated settings. It is deducted from commission payable, not from salary.

13. Doctor Salary Tax

Doctor salary profile has its own salary-tax configuration. Salary tax and commission tax do not merge.

14. High-Cost Medicine

Management configures a threshold. Expensive medicine requests can require attendant confirmation and/or Admin/Super Admin credential authorization. Pharmacy cannot dispense while authorization is pending.

15. Doctor Discharge

Admission user starts discharge, but doctor must enter clinical authorization username/password and complete discharge summary. The system records the doctor identity and time.

- Doctor username/password is validated, not merely typed as a name.
- Summary captures diagnosis, treatment, condition, medicines/instructions and follow-up.
- Doctor ID/name/time are stored.

16. Billing Pending

After clinical discharge, status becomes Clinically Discharged - Billing Pending. Front Desk finalizes financial closure; Admission user still does not collect cash.

17. Final Self-Pay Billing

Front Desk freezes/finalizes department invoices, applies advances/partial payments and collects the patient remaining responsibility. Receipt allocations are posted to the related department invoices.

18. Panel Patient Verification

Panel patient is a permanent reusable record. Front Desk validates company/member status, tariff/coverage and required authorization reference before admission/encounter.

19. Panel Department Billing

Every department invoice separates Patient Share and Panel Receivable. Patient co-pay is collected by Front Desk; panel portion remains receivable until the panel actually pays.

Department	Patient Share	Panel Receivable
Hospital	configured	configured
Lab	configured	configured
Pharmacy	configured	configured

20. Panel Discharge

Doctor discharge is the same clinically. Financially, Front Desk clears patient share and confirms panel claim/authorization data. Panel receivable can remain open after patient leaves according to contract.

21. Outsourced Department Setup

Admin/Super Admin creates the department and links an Outsourced Provider with representative, payment terms, allowed payment methods, bank/cheque details, settlement cycle and optional withholding/tax configuration.

22. Provider Settlement

Front Desk collections feed department/provider ledgers. Authorized Billing/Admin/Super Admin can settle Full or Partial amounts. Settlement cannot exceed eligible realized payable.

- Settlement methods may include Cash, Bank Transfer, Cheque and configured Online/Other.
- Provider representative acknowledgement and voucher are required when policy enables them.
- Partial settlement keeps remaining payable open.

23. Panel + Outsourced Settlement

For a Panel case, provider settlement uses realized money only: patient share collected plus panel remittance actually received and allocated to that department.

24. Management Reporting

Management can view department billed vs collected vs panel receivable vs provider payable vs settled vs outstanding, along with doctor discounts, commission tax, salary tax, high-cost authorizations and user accountability.

25. Final v7.2 Rule

Patient experience may look like one hospital journey, but the system preserves clinical authority, payer responsibility, department ownership, central cash custody and settlement accountability as separate controlled layers.